Day 4: make the value visible

500 Eurasia B11 | 1 October 2026 | Two recordings, 3h 40m 24s

The whole day in one sentence

Choose customers with an urgent problem, show them the result they care about, then make the path to that result easier and measurable.

1. The same product can be essential or optional

Urgency depends on the customer and their situation. Start with buyers who have a pressing problem and a strong reason to pay for solving it, not everyone who likes your product.

2. Your product is the whole path to value

Onboarding, guidance, handoffs, confidence and follow-up matter alongside software. A missing human step can block a customer even when all the features exist.

3. A demo is evidence, not a tour

Confirm the buyer's top problem. Show the most relevant result early. Tie each action to a benefit, ask specific questions, and agree who does what next and when.

4. Measure progress, not just activity

For each important step, track who reached it, who moved forward, how long that took and why others stalled. Use that evidence to choose your next experiment.

MediMind's practical takeaway

Do not sell the whole hospital future in the first ten minutes. Demonstrate one painful hospital workflow working better, then earn a clearly scoped parallel-system evaluation. The larger operating-system vision explains where this can lead; the first test supplies the proof.

Fast path: read this page, then hospital journey (p. 6), demo rehearsal (p. 11) and assignment checklist (p. 15). Use the full transcript PDF for the surrounding discussion.

Find the useful moments

S1 and S2 identify recordings. D1 and D2 identify decks. Times restart for each recording.

Source	Content	Rewatch anchors (approx.)
S1 · 2h 9m 51s Nicholas Ho	Product perception; urgency and return; customer journeys; activation; focused entry offers; metrics.	00:24 main lesson 00:35 urgency examples 00:55 customer matrix 01:00 AARRR 01:05 Flash case 01:40 golden gap 01:53 homework 01:55 seasonality
S2 · 1h 30m 33s Brianna Salinas	Sales demos: preparation, structure, buyer questions, objections, next steps and practice.	00:01 demo goal 00:14 AI demo prep 00:26 stakeholders 00:32 transitions 00:40 objections / price 00:46 examples 00:58 practice task
D1 · 52 slides	Sales Demos - Brianna Salinas. First link supplied.	5-7 engagement diagrams 17 / 49 demo canvas 21-32 script 51 assignments
D2 · 39 slides	Product Optimization & Conversion - Nicholas Ho. Second link supplied.	5-14 urgency / value 16-30 journeys 31-35 golden gap 37 metrics; 38 homework

Three evidence labels

Lecture takeaway: a paraphrase of the teaching.
Reported case: an instructor's example, not an independently audited result.
MediMind application: our proposed adaptation, not a lecturer's assessment or a measured company result.

What is included

Both unique recordings and both decks. Matching M4A files duplicate the MP4 audio and are not counted again. S1's main lesson starts around 00:24; S2's teaching ends around 01:02. The guide omits administration, energizers and unclear room talk; the transcript retains all recognized speech. Rewatch times are navigation aids, not exact quotation boundaries.

The recording order differs from the link order

Product optimization is Session 1, although its deck was the second link. Sales demos is Session 2, although its deck was the first link. Use both the session number and time when returning to a moment.

Sell relief from a real problem

Lecture takeaway | Nicholas Ho | D2 slides 4-14

Urgency × value to this buyer

A useful customer is not simply someone who could use the product. They have a problem important enough to address now, and the improvement is valuable enough to justify the effort and price.

Concept	Plain-language meaning	What to ask
Painkiller	Solves a pressing problem with a visible, near-term benefit.	What breaks, costs money or wastes time if you do nothing?
Vitamin	Helpful improvement, but easy to postpone.	Why would this become a priority now rather than later?
A spectrum	The same product can be urgent for one customer and optional for another.	Who needs this most, in which situation?
Relative value	Context and financial capacity affect perceived price and return.	What is the problem worth solving for this buyer?

The matrix

Put **urgency** on one axis and **customer return** on the other. Prioritize the high-urgency, high-return corner. A segment with impressive size but little immediate need may be a poor first target.

Why the pen and bag examples matter

A pen is not inherently essential or optional; the situation changes its importance. A luxury purchase can meet a social or emotional need, not only a functional one. The cinema example shows how context changes willingness to pay. These are teaching illustrations, not pricing benchmarks.

Two extra points from the spoken discussion

Timing: look for built-in urgency and seasonal triggers rather than manufacturing pressure; track enough history to see patterns.
Proof: a buyer's pain can be real even when confidence in your solution is low. Diagnose whether the barrier is priority, product fit or trust before changing the pitch.

MediMind application

'AI is the future' may attract interest without creating action. 'Your team is re-entering the same information, and we can test an alternative alongside your current system' connects the future to a problem the buyer can recognize today. Confirm that pain before using it in the demo.

MediMind's customer-priority matrix

Completed assignment draft | Hypotheses to validate in discovery, not measured market research

Customer situation	Urgency / potential value	Priority and evidence needed
Private hospital with recurring cross-team rework, an accountable leader and an upcoming decision	High / high if the problem is frequent and measurable	**First target.** Confirm the broken workflow, its frequency, current effort and a decision date.
Hospital group adding sites or standardizing operations	Medium-high / potentially high across sites	**Selective target.** Verify the immediate trigger; begin with one site and workflow, not a group-wide promise.
Hospital curious about AI, but no urgent problem, owner or change window	Low / uncertain	**Nurture.** Offer learning and revisit a genuine trigger. Do not fund a bespoke build just because they are interested.

Ideal customer profile is not the same as a persona

The **ideal customer profile (ICP)** describes the hospital situation worth pursuing. A **persona** describes a person involved in the decision. Within one hospital, the CEO may want control and financial value, clinical leaders need workflow fit and safe review, and IT needs a manageable evaluation and implementation path.

Five qualification questions

1. Which recurring workflow causes the most avoidable work?
2. What happened recently that makes fixing it important now?
3. Who owns the problem and who can approve an evaluation?
4. What would count as a meaningful improvement?
5. Can clinical, operations and IT reviewers participate in a dated comparison?

Decision rule

Advance when pain, an accountable sponsor and a feasible next step are confirmed. A warm introduction opens a door; it does not establish fit, budget or readiness. Record unknowns instead of filling them with optimistic assumptions.

The product is a path, not a menu

Lecture takeaway | D2 slides 16-35 | From the customer's starting point A to desired result B

Ask one organizing question

What must the customer actually do to get the outcome they want? Map that path first. Then decide which features, people, instructions and evidence help them move along it.

AARRR stage	Question it answers	Hospital interpretation
Acquisition	How do suitable people find and engage with you?	Qualified hospital, credible connection, discovery and relevant demo.
Activation	When do they first experience useful value?	Reviewers successfully test the agreed workflow and recognize its usefulness; not just receiving a login.
Retention	Do they return and continue getting value?	Repeat meaningful use by the intended team; distinguish evaluation use from paid adoption.
Referral	Do they become advocates?	An earned introduction or permission to use a reference, not assumed endorsement.
Revenue	Do they pay and continue paying?	Signed paid agreement, collection, ongoing use and renewal tracked separately.

The golden gap: focus within the bigger journey

In the recording, Nicholas defines this as choosing **one valuable part of the customer's A-to-B process** to solve and sell especially well. His restaurant-training example narrows to teaching staff to take orders and upsell rather than selling every training function at once. It is a focused entry offer worth testing, not a universal rule to abandon an end-to-end product.

Five blind spots from the lecture

Building a full-stack solution immediately; starting with features instead of a path; asking only what features users want; ignoring data; and failing to execute chosen ideas consistently.

Manual help can be part of the product

In the spoken Flash case, staff physically help merchants get started; another restaurant-software example uses manual onboarding. The lesson is to compare cost with customer results, not to automate by default. For MediMind, a guided first test may be more valuable than a self-service login that nobody uses.

Important distinction for MediMind

A broad hospital platform can remain the strategy while the first evaluation is narrow and testable. You do not need to demonstrate every department to establish that one important workflow can improve.

The complete hospital journey

MediMind application | A simple operating flow with separate buyer and delivery milestones

Stage	What must be true to advance	Owner / evidence
1. Find a fit	A named hospital matches the chosen customer situation and has a plausible trigger.	Founder / account record and reason for fit
2. Find a route in	A mediator, existing connection or potential champion can reach the relevant decision team.	Founder / introduction or direct-contact path
3. Discover and qualify	Confirm the painful workflow, sponsor, reviewers, impact and buying process.	Founder + hospital sponsor / discovery notes
4. Demonstrate relevant value	Show that workflow, ask what changed for the buyer, surface objections and agree the next step.	Founder + product lead / dated next meeting
5. Make the evaluation ready	Agree scope, baseline, success criteria, reviewers and decision date; complete necessary data and security approvals.	Hospital sponsor + IT + MediMind / readiness checklist
6. Deliver the parallel system	The agreed test environment is usable and the hospital confirms it can begin the comparison.	MediMind delivery lead / acceptance and ready date
7. Complete the evaluation	Named reviewers test the agreed scope and discuss evidence in a decision meeting.	Hospital reviewers + founder / proceed, revise or decline
8. Contract and adopt	Agree paid scope, rollout, training and responsibilities; verify sustained use and payment.	Buyer + delivery lead / agreement, use and collection
9. Retain and expand	Review realized value, resolve friction, renew, then earn expansion or introductions.	Account owner / value review and renewal evidence

Flow: qualified hospital → connection → discovery → relevant demo → evaluation ready → parallel system delivered → evaluation completed → paid adoption → retention / expansion. If a gate fails, record the reason and next action; do not silently advance the stage.

Permission, delivery and value are different events

Approved data access is a readiness milestone. A working system is your delivery milestone. A completed comparison is the hospital's evaluation milestone. One does not prove the next.

One metric, with useful diagnostics

MediMind application | Keep the current bottleneck visible without building a complicated dashboard

Primary metric: completed new-hospital evaluations

Count unique non-anchor hospitals that complete a defined parallel-system evaluation in a rolling 28-day window. Review weekly. Require named reviewers, evidence from the agreed test and a recorded proceed / revise / decline decision. Count each hospital's first qualifying evaluation once.

Supporting measure	Precise definition / use
Qualified opportunities	Hospitals with confirmed fit, pain, sponsor and feasible next step. Exclude raw contact lists.
Demo → ready evaluation	Number reaching the readiness gate / qualified hospitals completing a demo in the same tracked cohort.
Ready → delivered	Count and rate of evaluation-ready hospitals receiving the agreed usable environment. Track delivery time separately.
Delivered → completed	Number completing the agreed review / delivered evaluations in the same cohort. Show pending cases and age.
Time to first useful result	Calendar days from evaluation ready to the first reviewer-confirmed useful workflow result. Also track earlier qualification-to-ready delay.
Completed → paid	Number with a signed paid agreement / completed evaluations in the same cohort. Track cash collection separately.
Evaluation effort	Founder, engineering, implementation and support hours per evaluation, plus open defects and rework.

Prevent misleading numbers

Always show counts beside percentages. Keep anchor-partner activity separate. Use a stable hospital ID; a multi-site group is not automatically many customers. Keep unresolved evaluations visible so faster completed cases do not hide stalled ones.

Time frames are planning assumptions

Your proposed two-week delivery window starts only after the readiness prerequisites are complete and capacity is confirmed. It is not a two-week promise from first contact. Agree a separate testing and decision window with the hospital; the rolling 28-day metric is a reporting period, not a guaranteed sales cycle.

Recommended record: hospital ID, segment, source/introducer, sponsor, stage-entry dates, baseline, test scope, reviewers, decision date, next action/owner, blocker, outcome and delivery effort. No patient information belongs in the sales tracker.

Choose a valuable point of focus

Completed golden-gap and experiment-backlog draft | Proposals, not validated findings

Candidate golden gap: one handoff without repeated work

For a hospital where staff repeatedly gather and re-enter information, test a focused promise: make one agreed cross-team handoff easier to complete and verify. Map the current task, demonstrate the alternative and measure effort plus quality. This is a hypothesis to validate, not an established MediMind result. Keep the broader hospital OS as the expansion path.

Separate adoption-gap hypothesis: an interested hospital may still lack scope, reviewers and a decision process. The parallel evaluation helps test the focused product promise; it is not itself the definition of the golden gap.

Experiment	What changes	Evidence to review
A. One-page evaluation agreement	Before building, name one workflow, success criteria, hospital reviewers and a decision date.	Do more qualified demo participants become evaluation-ready? Which condition still blocks them?
B. Outcome-first demo	Start with the buyer's highest-priority workflow, not a product menu. Ask a specific value question.	Does the buyer recognize the improvement and agree a dated next step? Capture their words.
C. Guided first test	Schedule a short working session in which the reviewer performs the task, rather than only receiving access.	Time to first useful result, task completion and assistance needed.
D. Evidence-led decision review	Use the baseline and test results in the already-booked review meeting.	Delivered-to-completed rate, unresolved objections and proceed / revise / decline reasons.

The product test inside the evaluation

Ask intended users to perform the same defined handoff in the current process and the alternative. Compare time, repeated entry, missing information and assistance required. Confirm that faster work remains correct. This tests the golden-gap hypothesis; the table above tests how hospitals reach and complete that comparison.

Example first test

Apply the one-page agreement to the next three qualified opportunities, subject to capacity. Review weekly for two weeks: can participants name reviewers and book a decision date? Add one owner, dates and a decision rule to Day 3's experiment board. Small samples provide directional learning, not statistical proof or guaranteed results.

Guardrails

Do not increase free-build volume until delivery capacity is clear. Do not trade away data controls, patient safety or honest claims to improve a conversion rate. Any clinical AI capability shown must reflect what is actually available and its required human review.

A demo earns the next commitment

Lecture takeaway | Brianna Salinas | D1 slides 2-32

Five jobs of the demo

Make the pain concrete. Show the desired outcome. Provide proof you can deliver. Make adoption understandable. Earn a specific next step.

Before the call

Map the attendees and what each values. Ask your contact who else should join. If senior or unfamiliar stakeholders attend, request a short preparation call. Have a populated, tested demo environment and a backup recording or screenshots. Know the relevant buyer metrics.

A useful sequence

Confirm time and agenda → introduce new participants → recap and verify the problem → choose the top priority → show the most relevant value moment → show only supporting features tied to benefits → discuss adoption, objections and pricing where appropriate → agree next action, owner and date.

Keep the buyer in the conversation

The engagement diagrams contrast an excited seller with a distracted buyer. Break up the monologue with specific questions about fit and impact. Ask whether the workflow solves the problem discussed, what it would change, and what would prevent adoption.

AI demos: prepare without pretending

The recording recommends rehearsed inputs, familiar data and backups; it also allows mockups when a product is not yet built. **MediMind adaptation:** label recorded examples, mockups and future capabilities honestly. A prepared success is not a reliability study. Acknowledge failures and use the evaluation to test representative cases, not only the showcase example.

Price is optional; understanding the reaction is not

An enterprise proposal may need a separate conversation. If you give a price, state it simply and pause. If it feels expensive, ask what the buyer is comparing it with before explaining value or changing the offer.

Avoid	Do instead
Long company introduction	Give only the context needed to trust the relevant demonstration.
Login / empty dashboard first	Open at the useful result or workflow; show realistic, prepared data.
Feature inventory	Explain why this action matters to this buyer.
Only 'Any questions?'	Ask about impact, workflow fit and specific remaining concerns.
'We'll follow up'	Confirm a concrete next step, responsible people and timing.

Hospital-specific adaptation: use realistic synthetic or appropriately authorized de-identified demonstration data. The deck's 'real data' advice means a credible, populated demo; it is not permission to disclose patient records.

MediMind's completed demo canvas

Assignment draft | Adapt the canvas to the specific hospital after discovery

Working value proposition

See how your hospital could work with one connected system: test a tailored alternative alongside your current software before deciding to switch.

Canvas field	MediMind draft
ICP / buyer	Hospital with confirmed cross-team workflow pain, an accountable CEO or operator, and clinical / IT counterparts able to evaluate.
Customer pain	Staff repeating work, chasing missing information or struggling with handoffs. Choose only pains this hospital has confirmed.
Pain → demonstration → benefit	Show the agreed task across its handoff; compare steps and effort with the current process; ask whether the result is useful.
Moment of excitement	The reviewer recognizes their own recurring problem and sees a credible simpler way to complete the task. Select a working capability, not a roadmap animation.
BANT	Budget: current spend and decision limits. Authority: approver and evaluators. Need: priority workflow. Timeline: trigger, evaluation window and purchasing process.
Potential objections	Disruption, staff adoption, IT workload, data handling, uncertain benefit, price scope, missing functionality and confidence in delivery.
Questions still unanswered	What must be demonstrated? Who judges it? What can block an evaluation? What happens if the comparison succeeds?
Pricing / next step	Explain the free, scoped parallel evaluation and any written pricing terms. Agree an evaluation-planning meeting with sponsor, clinical / operations and IT owners.

BANT means budget, authority, need and timeline. Use it to locate unknowns, not as an interrogation. A friendly champion may influence the decision without having signing authority.

Prepare three versions, not three products

CEO: control, staff effort and economics. Clinical leader: usable information, workflow fit and human review. IT lead: feasibility, data controls, integration work and support. Keep one shared outcome; change the evidence emphasized for each participant.

A 10-minute demo rehearsal

MediMind practice draft | Not a real meeting-length rule or proof that every capability is available

Time	Say / show	Question or commitment
0:00-1:00	'You told us your team loses time on [confirmed workflow]. I will show an alternative, then we can decide whether it is worth testing at your hospital.'	'Is that still the right priority? What else must we cover?'
1:00-2:00	Recap the current steps and the baseline the hospital supplied. Explain the exact comparison you will show.	'Which part causes the most repeated work?'
2:00-4:30	Demonstrate the strongest working result first using prepared, safe data. Complete the relevant task, not a tour of departments.	'Does this address the problem you described? What would change for your team?'
4:30-6:00	Show one supporting handoff or AI-assisted task if available. Explain inputs, output and human review; let the buyer inspect it.	'Where would this fit, and what would you need to verify?'
6:00-7:30	Explain a separate parallel evaluation, agreed scope, reviewers, prerequisites and how the current operation remains separate.	'Who would need to take part for this comparison to be meaningful?'
7:30-8:30	State the evaluation offer and pricing terms plainly. Separate what is available now from future development.	'What concerns would stop you from evaluating this?'
8:30-10:00	Recap the confirmed value and remaining unknowns. Propose a scoped planning session with the required hospital counterparts.	Agree the next action, people and date before ending.

The strongest transition

'Instead of asking you to believe a promise, let's agree what better looks like and let your team compare it.'

If two founders join the demo

The lecture recommends one person controlling the demonstration and its pace, while the other listens, takes notes and asks useful clarifying questions. Agree those roles beforehand so the buyer is not watching two people direct each other's clicks.

The instructor asks for a **chosen 10-minute segment**, not a whole demo squeezed into ten minutes. This compact simulation is our practice option; choose the portion you most need to rehearse. A real meeting needs room to listen. Do not force the script past unresolved questions or claim untested features work.

Make the offer easy to understand

MediMind application | Buyer-facing draft plus the conditions needed to deliver honestly

A concise evaluation offer

'We will build an agreed parallel version for your hospital to test, free and without a purchase commitment. Compare it alongside your current system before deciding whether to switch.'

Pricing-match wording, if the offer is active

'For our next 10 eligible hospital clients, we will match the current software price within the agreed scope.' Define eligibility, comparable costs, exclusions, contract period and available places in writing. This is the founder's proposed offer, not an independently verified active promotion.

How to explain AI without losing credibility

'First, we connect the workflow and make everyday work easier. Then we add the AI assistance your team can use and verify. That connected foundation supports more advanced AI over time.' Label future capabilities as future work; do not present autonomous medical decisions as an established safe outcome.

Buyer concern	Useful response / next evidence
'We cannot disrupt operations.'	Explain what the separate evaluation does and does not touch. Confirm boundaries and prerequisites with IT.
'Our staff will not use it.'	Name reviewers, give a guided first task, observe use and record friction rather than just sending logins.
'What improvement will we get?'	Agree the baseline and comparison criteria. Measure the result; do not invent a guaranteed saving.
'How much work is this for us?'	State the hospital inputs, approvals, meetings and review time before the build starts.
'What happens after the free test?'	A dated review: proceed, revise the agreed scope or decline. A paid rollout needs its own agreement.

If the buyer says it does not solve the problem

Pause and ask why. The spoken lesson distinguishes poor explanation, changed priorities, weak discovery and a real product mismatch. Reconfirm the desired outcome; do not force the script or treat agreement as more important than fit.

'Free' refers to the agreed evaluation charge, not zero hospital effort. 'No purchase commitment' does not remove the need for evaluation, confidentiality or data-handling terms. Future development promises must state scope and service commitments before they become contract claims.

Prove value without inflating it

MediMind application | A small evaluation scorecard, not an ROI or clinical-outcome guarantee

Value area	Practical comparison	Important boundary
Staff effort	Time and repeated-entry steps to complete the same defined task before and during the test.	Same task and conditions; record assistance, sample size and exceptions.
Handoff reliability	Missing items, corrections and successful completion of agreed handoffs.	A cleaner workflow is not automatically proven improvement in clinical outcomes.
Usability	Can intended reviewers finish the task? How much help is needed?	A founder-led demonstration is not independent user completion.
Business value	Eligible missed charges, avoidable rework or capacity constraints identified and validated by the hospital.	A suggested opportunity is not collected revenue. Avoid double-counting benefits.
Delivery burden	Hospital and MediMind time, unresolved defects, setup work and support needs.	Price matching alone does not prove lower total cost.

Simple, honest arithmetic

Estimated staff time released = comparable task volume × measured minutes saved / 60. If the hospital supplies an hourly cost, show the implied capacity value separately. It becomes cash saving only if spending is actually avoided or reduced. Additional revenue must be verified and collected; it is not created by multiplying a hopeful percentage.

Turn a real result into credible proof

The spoken lecture connects customer results to future acquisition: deliver value → document the evidence → create an approved case study → make the next sale easier. For MediMind, record the initial problem, test scope, measured result, limits and hospital-approved wording. Do not treat an evaluation as permission to name a hospital publicly.

Keep a quality counterweight

If a task gets faster but produces more errors, greater review burden or an unsafe handoff, it has not demonstrated the intended improvement. Record quality and effort alongside speed.

What success sounds like

'The named reviewers completed the agreed comparison, saw [documented result], identified [remaining gaps], and agreed [next action].' This is much stronger evidence than 'They loved the demo.'

The cases worth remembering

Reported teaching examples | Use the mechanism, not another company's numbers as your forecast

Case	What the lesson demonstrates	MediMind translation
Flash	Signing a merchant is not enough: onboarding, a first transaction and repeat transactions form the path to value. Track time between milestones and support the next behavior.	Separate hospital interest, evaluation readiness, delivery, first useful test and completed review.
Hotel booking	Every step between searching and completed checkout can help or lose the customer. Add step metrics and test the bottleneck.	Map the actual hospital decision path before adding outreach volume.
Coaching program	First attendance, ongoing use and renewal depend on more than access to content or tools.	A login or training invitation is not evidence of adoption.
Fashionlab	The instructor uses a customer message to illustrate how hands-on onboarding builds confidence to produce useful results.	Guide hospital reviewers through their first meaningful task and observe where they need help.
Marvin demo	Start with the buyer's research problem, show centralization and useful retrieval, then connect supporting features to outcomes.	Let the hospital's priority determine what appears first; show AI in the context of a useful workflow.
Sparrow demo	Adapt to HR's operating pains and multiple stakeholders; show a relevant process and end with rollout questions.	Align CEO, clinical / operations and IT interests around one evaluation outcome.

Two source details to treat carefully

Flash journey boxes show 5 / 50 / 500 transactions, while some KPI captions use 100 / 1,000. Nicholas explicitly notes this mismatch around S1 01:21-01:22. Treat the numbers as illustrations, not universal thresholds. The coaching slide also reuses 'merchants' in one KPI. Define your own terms and milestones.

Flash: the spoken operating lesson

Nicholas describes incentives tied to meaningful merchant activity, one shared account record, named owners and hands-on onboarding. The case uses a 48-hour onboarding target and transaction milestones; these are Flash-specific, not hospital targets. He checks whether early wins persist. The closing Q&A; treats incentives as variable costs whose return must be measured. Focus on activation before optimizing every AARRR stage.

Flash: look inside seasonality

In S1 around 01:55-01:58, a reported seasonal slowdown led the team to compare customer verticals and test logistics customers plus transaction incentives. The lesson is to find where activity continues, not assume every segment slows together. This is a company-specific case, not a universal claim about Ramadan or hospital buying cycles.

The chart in the Flash deck reports growth but does not isolate which intervention caused it. Script examples containing large revenue or time-saving claims are illustrative placeholders, not substantiation for MediMind's copy.

Your assignment checklist

What the decks explicitly ask for, and where the completed MediMind drafts are in this guide

Spoken clarification: Nicholas says the product exercises are not enforced and the golden-gap exercise is optional (S1 around 01:53). Brianna also makes the canvas optional; the assignment is to practice a selected demo segment, stating the ICP and the part you are practicing (S2 around 00:58).

Course requirement	Your draft / next action
Product: map the ICP matrix D2 slide 38	Page 4 ranks three hospital situations by urgency and potential value. Validate the ranking in real discovery calls.
Product: map the AARRR flow and assign KPIs D2 slide 38	Pages 5-7 connect the hospital journey, stage gates and metric definitions. Start with one shared account tracker.
Product: update the experiment backlog D2 slide 38	Page 8 gives four adoption tests plus a product-workflow comparison. Select the relevant test, add an owner and dates, and use Day 3's board.
Product: highlight the golden gap D2 slide 38	Page 8 proposes a focused cross-team handoff with less repeated work. Confirm it is valuable to the chosen hospital; distinguish this product focus from evaluation-process friction.
Demo: finish outline and talk track D1 slides 50-51	Pages 10-12 provide the canvas, 10-minute outline, offer and objection handling. Replace placeholders with one hospital's actual discovery evidence.
Demo: deliver 10 minutes and get feedback D1 slide 51	Rehearse the flow, record it and ask whether the pain, outcome and next step were clear. Check live capabilities first.
Demo: adapt for another ICP and practice D1 slide 51	Change the priority, opening evidence and questions for a different hospital situation or stakeholder; do not merely change the logo.

Historical timing, not a new deadline

The slide says 'today' finish the outline, 'tomorrow' present, and 'next week' adapt. In this 1 October session, tomorrow meant 2 October. The opening audio describes roughly 15 minutes per company: about 10 minutes to practice a chosen demo segment and five for feedback. The full 10-minute outline on page 11 is our suggested rehearsal, not a requirement to show everything.

What this pack does not claim

The drafts are ready to discuss and customize. They are not evidence that customer interviews, hospital tests, a demo rehearsal, contracts or experiments have already happened. Those actions remain yours to carry out.

Metrics beyond the first evaluation

Lecture takeaway and MediMind interpretation | D2 slide 37

Metric	Meaning	When it helps MediMind
Churn	Paid customers leaving. A lagging result; declining meaningful use can be an earlier warning.	After paid adoption. An evaluation ending without purchase is conversion loss, not paid-customer churn.
Time to results	How long it takes the customer to experience a useful outcome.	During evaluations and onboarding. Define the start and result clearly before measuring.
Customer Effort Score (CES)	How easy or difficult a specific task felt to the customer.	Immediately after a reviewer completes the agreed task; use the same scale and record why.
Customer Satisfaction (CSAT)	Satisfaction with a defined interaction or experience.	After onboarding or support. Ask about that event rather than an undefined overall impression.
Net Promoter Score (NPS)	Likelihood to recommend; an advocacy signal, not a direct measure of task success.	After meaningful experience. With few hospitals, read individual reasons and show sample size.

Start with the decision, then choose the tool

The deck shows Google Forms, Hotjar, Mixpanel and Google Analytics as examples. You do not need all of them. A small hospital pipeline can begin with consistent stage dates, task observations and structured notes. Add instrumentation only when it answers a real question.

Usage is not the same as a result

The recording explicitly separates 'how much they used it' from 'how soon it helped'. Combine appropriate task evidence with a direct question about the outcome. A login, many clicks or a completed setup can be a leading signal without proving customer value.

Keep sensitive data out of generic analytics

Do not send patient content, identifiers or clinical screens to ordinary marketing trackers or session-replay tools. Design any product measurement around appropriate controls and minimal necessary data. Tool logos in a lecture are not an approval for clinical use.

The weekly review question

Which hospital is stuck, at which step, for what reason, and what is the smallest responsible change that could help it progress?

Turn the lesson into next week's work

Suggested action plan | Not a lecturer-set schedule or a guaranteed conversion target

When	Concrete output
Day 1	Choose the first hospital situation. Review existing opportunities against pain, sponsor, urgency and feasible next step.
Day 2	Map the hospital's workflow and choose one valuable problem to solve first. Separately map the sales journey and identify its largest delay.
Day 3	Fill the demo canvas for one real hospital. Prepare one working workflow, safe demonstration data and a backup.
Day 4	Rehearse the 10-minute demo. Remove anything that does not explain the buyer's problem, useful result or next step.
Day 5	Use the tailored demo and one-page evaluation proposal in an appropriate qualified conversation. Record objections and next-step evidence.
End of week	Review stage counts, delays, effort and learning. Decide which test to continue, change or stop. Carry pending hospitals forward honestly.

Five questions you should be able to answer

Who should buy first? A hospital with urgent, valuable and addressable pain.
What should the demo prove? A useful outcome for that buyer.
What is activation? First meaningful value, not mere access.
What is the golden gap? One valuable part of the customer's journey you choose to solve and sell especially well.
What ends a good demo? A mutually agreed next action, owner and date.

The sentence to keep

Make the next step smaller, the proof clearer, and the customer's result easier to reach.

A completed evaluation creates learning even when the answer is 'not yet' or 'no'. Keep positive decisions and paid conversions visible alongside completion so the primary metric cannot reward low-quality evaluations.

Sources, coverage and limitations

Private learning notes | Prepared 8 October 2026 | Original recordings remain unchanged

Recordings

S1: GMT20261001-054757_Recording_1920x1080.mp4 - 2h 9m 51s.
S2: GMT20261001-090151_Recording_1920x1080.mp4 - 1h 30m 33s.
Original folder: Desktop / Recordings / week2day4. Matching audio files were treated as duplicates.

Decks

D1: Sales Demos - Brianna Salinas, 52 displayed slides.
D2: Product Optimization & Conversion - Nicholas Ho, 39 displayed slides.
Both were reviewed through accessible slide text and relevant visual diagrams. Download / copy / print were disabled in the viewer, so this pack keeps links and original study notes rather than exported deck copies.

How the pack was prepared

Local automatic speech recognition, comparison with the decks, sampled on-screen slide review, and practical synthesis. The guide distinguishes lecture content from proposed MediMind applications. The separate transcript retains recognized wording with session-local timestamps and no invented speaker attribution.

What to verify before reusing a claim

Names, accents, room discussion and overlapping speech can be mistranscribed. The study guide is not a certified verbatim record. Case-study numbers, teaching examples and product claims were not independently audited. MediMind proposals require actual product, commercial and customer validation.

Folder guide

01 PDF: this concise study guide.
02 PDF: complete automatic transcripts.
Transcripts: searchable TXT, timestamped SRT and structured JSON for both sessions.
Source decks: source links and access notes.
Working files: reusable notes, processing files and verification records.